

Patient ID: TaylorPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 658/720h (91%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30

■ HR ■ Breathing

Elevated Breathing Apr 12 to Apr 12 (1d)	Elevated Breathing Apr 30 to Apr 30 (1d)	Low Heart Rate Apr 05 to Apr 05 (1d)	Low Heart Rate Apr 16 to Apr 16 (1d)	Low Heart Rate Apr 23 to Apr 23 (1d)	Low Heart Rate Apr 26 to Apr 26 (1d)
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Last 24 Hours

Vital	Avg	Min	Max
Heart Rate	60 bpm	44 bpm	92 bpm
Breathing	16 brpm	10 brpm	32 brpm

6 episodic events Detected, spanning 6h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
6	6h	Low Heart Rate	<1	91%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 05	8 AM to 9 AM	1h	1	Low Heart Rate	42	40	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 16	3 AM to 4 AM	1h	1	Low Heart Rate	44	40	51	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 23	4 AM to 5 AM	1h	1	Low Heart Rate	43	39	47	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 26	10 PM to 11 PM	1h	1	Low Heart Rate	45	39	52	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 12	9 AM to 10 AM	1h	1	Elevated Breathing	29	29	29	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 30	6 AM to 7 AM	1h	1	Elevated Breathing	26	15	32	Check SpO2 and lung sounds; assess for fluid overload or early infection

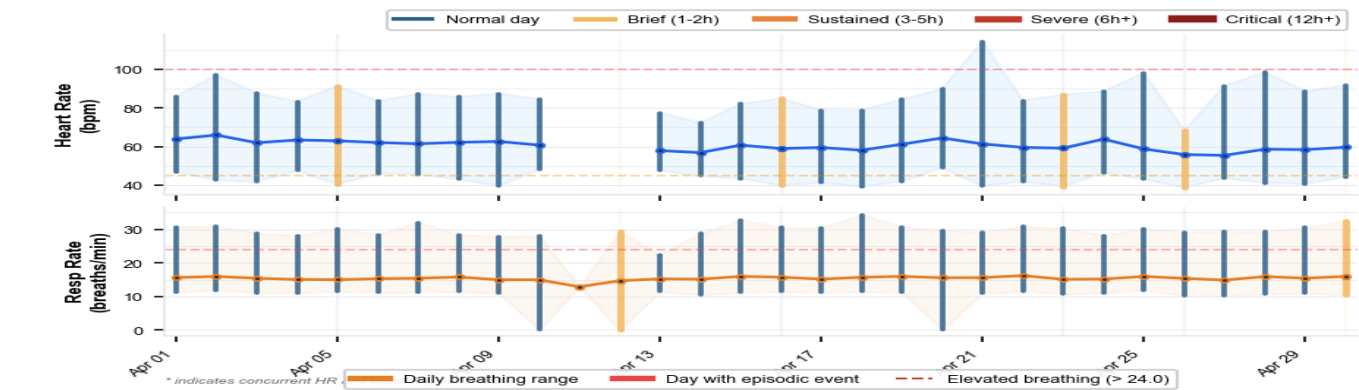
The P5 to P95 heart rate spread was 20 bpm (51 to 72), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 29 brpm, peak 29 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 42 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 6 unstable phases with 20 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

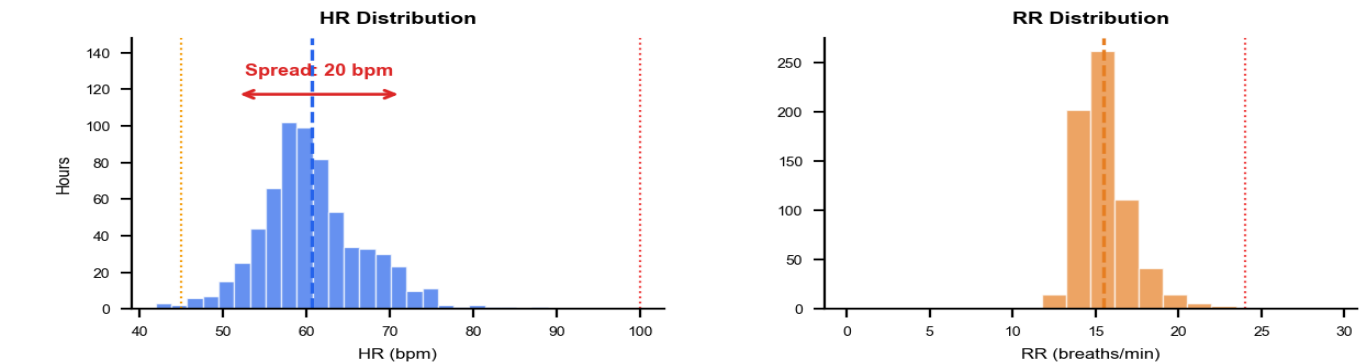
Clinical Pattern Observations:

- 1. High variability: HR spread 20 bpm (51 to 71).
- 2. Clustered pattern: Episodic events on 6 of 30 days (20%).

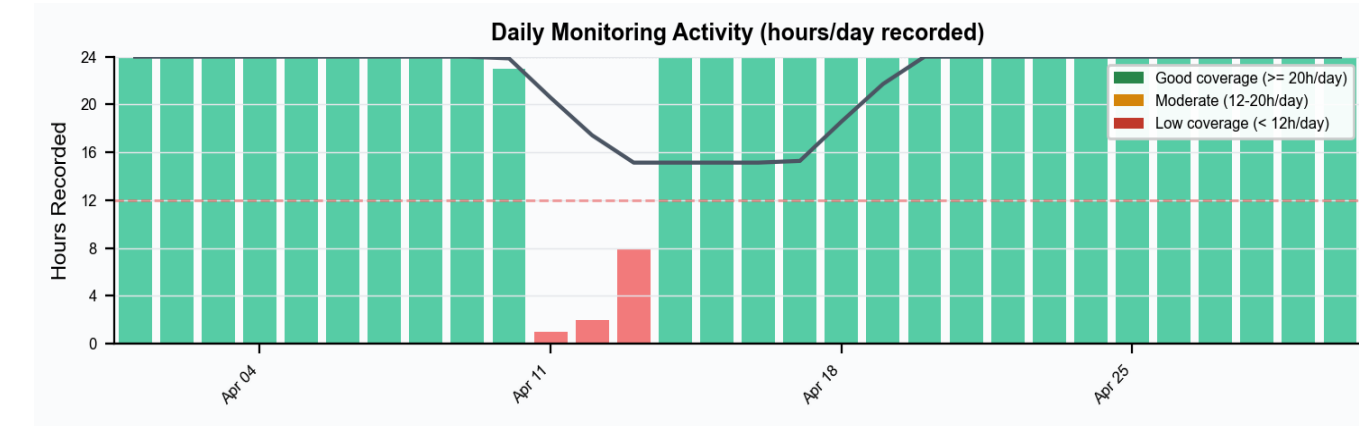


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.